INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Cisplatin, lomustine and vincristine (CLV)

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
42 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00806
Version 2

42 days: lomustine once + cisplatin intravenous (IV) D1; vincristine IV D1, 8 + 15.

Treatment stages

01 **Recovery after craniospinal radiotherapy**
Before CLV

02 **Cisplatin, lomustine and vincristine (CLV)**
42-day cycle

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Cisplatin, lomustine and vincristine (CLV)
National Cancer Control Programme (NCCP) 00806

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Fever, shivering or suddenly feeling unwell needs urgent advice, including on treatment rest days. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. Follow the emergency instructions in any established seizure rescue plan. Call promptly for fever, bleeding, new cough or breathlessness, persistent vomiting or reduced urine. Report hearing, nerve or bowel changes before the next infusion.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

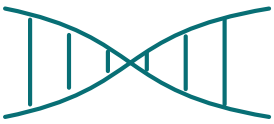
How your medicines work

CLV combines three chemotherapy medicines after craniospinal radiotherapy for selected adults with high-risk medulloblastoma or another eligible embryonal tumour. It is part of a specialist treatment plan; it is not a glioma regimen.

ORAL CHEMOTHERAPY CAPSULE

Lomustine

CCNU



Damages deoxyribonucleic acid (the cell's genetic material) (DNA)

Lomustine is an oral nitrosourea chemotherapy that can reach the brain. It damages DNA and interferes with cell division. Its effects on healthy blood-forming cells can appear several weeks after the dose and can build up over successive cycles. This explains the long gap between doses and the need for continuing blood tests.

One scheduled dose → DNA damaged → Delayed blood-count effects possible

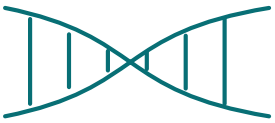
What to watch for

This is one dose on Day 1, not a daily medicine. If an extra dose is taken, get urgent advice even if you feel well. Keep blood tests after the dose and report fever, bruising, bleeding, new cough or breathlessness.

CHEMOTHERAPY INFUSION INTO A VEIN

Cisplatin

Platinum chemotherapy



Forms DNA links that disrupt cell division

Cisplatin forms chemical links within DNA, making it harder for cells to copy and repair their genetic material. It can damage tumour cells but also affect kidney function, hearing, nerves and blood-forming cells. Fluids, electrolyte checks and anti-sickness medicines are part of the treatment plan to reduce avoidable harm.

Platinum enters cells → DNA links form → Cell growth disrupted

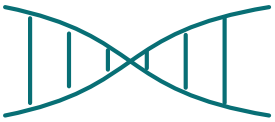
What to watch for

Report ringing or reduced hearing, numbness or weakness, reduced urine, persistent vomiting or difficulty drinking. Tell the team about fluid restrictions or heart and kidney conditions before hydration.

CHEMOTHERAPY GIVEN INTO A VEIN

Vincristine

Vincristine infusion



Disrupts the machinery used for cell division

Vincristine interferes with structures that cells use when dividing. It can also affect nerves controlling sensation, strength and bowel movement. This explains why tingling, weakness and constipation need early review even if blood tests are satisfactory.

Infusion into a vein → Cell division disrupted → Nerves and bowel may be affected

What to watch for

Report tingling, weakness, walking difficulty or constipation before the next dose. Tell staff immediately if the infusion site stings, burns or swells.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment and its purpose

Keep the exact diagnosis, regimen and treatment goal together.

What each medicine does

Lomustine: Lomustine is an oral nitrosourea chemotherapy that can reach the brain. It damages DNA and interferes with cell division. Its effects on healthy blood-forming cells can appear several weeks after the dose and can build up over successive cycles. This explains the long gap between doses and the need for continuing blood tests. Cisplatin: Cisplatin forms chemical links within DNA, making it harder for cells to copy and repair their genetic material. It can damage tumour cells but also affect kidney function, hearing, nerves and blood-forming cells. Fluids, electrolyte checks and anti-sickness medicines are part of the treatment plan to reduce avoidable harm. Vincristine: Vincristine interferes with structures that cells use when dividing. It can also affect nerves controlling sensation, strength and bowel movement. This explains why tingling, weakness and constipation need early review even if blood tests are satisfactory.

Your schedule

Start four to six weeks after craniospinal radiotherapy, once cleared by your team. In each 42-day cycle, lomustine is taken once on Day 1, cisplatin is infused on Day 1, and vincristine is infused on Days 1, 8 and 15. Eight cycles are planned, with individual safety review.

Evidence and uncertainty

The treatment strategy has a role in specialist multimodal care, but paediatric outcomes cannot be used as an adult survival estimate or applied to every embryonal tumour. The cited childhood study does not establish a numerical benefit for this exact adult high-risk population. Medulloblastoma and other embryonal tumours have different molecular and clinical contexts. Glioma trials and the uploaded glioma guide do not establish CLV efficacy. The neuro-oncology team must explain the evidence, uncertainties, treatment burden and alternatives for the individual diagnosis.

Consent and decisions

Discuss the exact diagnosis, treatment goal, evidence limits, alternatives and burdens; medicine-specific risks, monitoring, fertility and pregnancy precautions; any unlicensed supply or off-label use; and practical support. This schedule follows craniospinal radiotherapy (RT) and needs its own consent discussion.

CLV evidence: adult context and paediatric origins

The NCCP CLV regimen cites studies of cisplatin, lomustine and vincristine with radiotherapy in children with medulloblastoma.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your whole treatment course

Treatment stages

01 **Recovery after craniospinal radiotherapy**
Before CLV

02 **Cisplatin, lomustine and vincristine (CLV)**
42-day cycle

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

BEFORE CLV

Recovery after craniospinal radiotherapy

Start window: 4–6 weeks after radiotherapy (RT) **Safety and recovery review**

Record the actual RT finish date and specialist-approved chemotherapy start. Earlier surgery and RT have separate plans.

42-DAY CYCLE

Cisplatin, lomustine and vincristine (CLV)

Up to 8 cycles **42 days: lomustine once + cisplatin intravenous (IV) D1; vincristine IV D1, 8 + 15.**

Start four to six weeks after craniospinal radiotherapy, once cleared by your team. In each 42-day cycle, lomustine is taken once on Day 1, cisplatin is infused on Day 1, and vincristine is infused on Days 1, 8 and 15. Eight cycles are planned, with individual safety review.

Each start, visit, delay or dose change is confirmed by your team. The card records treatment and does not authorise another dose.

BASELINE → TREATMENT → FOLLOW-UP

Tests, symptoms and treatment decisions

Medicine-safety checks, tumour-tissue testing and response imaging answer different questions. Product-information additions are labelled. No universal blood tumour-marker panel or scan interval is implied.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00806 v2 · current regimen · NCI: medulloblastoma diagnosis, risk groups and treatment · patient education · HPRA: cisplatin Accord current product information · July 2026 · Lomustine medac current product information · January 2026

THE WHOLE TREATMENT JOURNEY

Cisplatin, lomustine and vincristine (CLV): treatment and checks

NCCP 00806: selected adults with high-risk medulloblastoma or another eligible primitive neuro-ectodermal tumour, including pinealoblastoma.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Next treatment / response	After a pause or stop
Bloods / medicine safety NCCP 00806	Blood count, kidney/liver tests; hearing assessment and creatinine clearance when clinically indicated; NCCP lung testing for patients at high risk. Current lomustine SmPC additionally advises baseline lung-function assessment. Current cisplatin product information advises baseline audiometry. Confirm recovery after RT and the individual hydration/ electrolyte plan.	Regular blood count, kidney/ liver tests, with hearing and lung assessment when indicated. Current lomustine SmPC advises weekly blood counts for at least six weeks after a dose and continuing lung assessment. Current cisplatin product information additionally advises weekly kidney/ liver tests, blood counts and electrolytes through treatment, with repeat hearing testing for symptoms. Review nerves and bowel function before vincristine.	Keep additional tests even on days without an anticancer dose.	Team reviews current and previous results, symptoms and dose history.	Agree continuing checks; lomustine effects may persist.
Neurology / medicines / function NCCP 00806	Record usual symptoms, medicines, daily function and support needs.	Review headache, seizures, speech, strength, cognition and any steroid plan.	Report new changes early; do not wait for an magnetic resonance imaging (MRI).	Discuss treatment tolerance, priorities and the next authorised dose.	Agree rehabilitation, symptom support and contacts.
Response / appointments NCCP 00806	Confirm exact diagnosis and baseline imaging.	Imaging and clinical review follow the specialist plan.	Record bookings, result-review contacts and questions.	Interpret scans with symptoms, treatment effects and medicines.	Agree the next imaging and clinical review.

What each check is for

1 **Recovery after craniospinal radiotherapy**

Before CLV

CLV combines three chemotherapy medicines after craniospinal radiotherapy for selected adults with high-risk medulloblastoma or another eligible embryonal tumour. It is part of a specialist treatment plan; it is not a glioma regimen.

Ask / act: Confirm the current phase, prescription, blood tests and support medicines.

2 **Cisplatin, Iomustine and vincristine (CLV)**

42-day cycle

CLV combines three chemotherapy medicines after craniospinal radiotherapy for selected adults with high-risk medulloblastoma or another eligible embryonal tumour. It is part of a specialist treatment plan; it is not a glioma regimen.

Ask / act: Confirm the current phase, prescription, blood tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking the treatment

Take Iomustine only once on the authorised Day 1. The pharmacist must confirm the total dose, capsule strengths, how to take it and anti-sickness timing. Do not open or crush capsules or take another dose to replace a missed or vomited one without advice. Cisplatin and vincristine are given into a vein. Day 1 includes fluids before and after cisplatin; later visits on Days 8 and 15 are for vincristine. The total visit is longer than the infusion itself.

STEP 2

Missed, vomited or held medicine

If a capsule dose is missed or vomited, call the oncology team or pharmacist before taking more. Do not double up or use leftover capsules. An infusion or capsule held for toxicity needs a new team decision.

STEP 3

Medicine and supplement review

Bring all prescription, over-the-counter and herbal medicines, vitamins and supplements to review. Include steroids, seizure medicines, anticoagulants and any recent antibiotics or antifungals. Tell the team before starting or stopping a medicine.

STEP 4

Capsule supply and handling

Lomustine is supplied as an unlicensed medicine in this NCCP protocol. Confirm pharmacy supply in advance. Keep the labelled pack away from children and do not open or crush capsules. Return unused chemotherapy to the pharmacy; contact staff if swallowing or handling is difficult.

STEP 5

Pregnancy, breastfeeding and fertility

Discuss fertility preservation and contraception before starting. Treatment can harm a pregnancy. Do not breastfeed during treatment. Ask for the written medicine-specific precautions and the required periods after stopping; these may differ between products.

STEP 6

Appointments and transport

Before starting: Blood count, kidney/liver tests; hearing assessment and creatinine clearance when clinically indicated; NCCP lung testing for patients at high risk. Current lomustine SmPC additionally advises baseline lung-function assessment. Current cisplatin product information advises baseline audiometry. Confirm recovery after RT and the individual hydration/electrolyte plan. During treatment: Regular blood count, kidney/liver tests, with hearing and lung assessment when indicated. Current lomustine SmPC advises weekly blood counts for at least six weeks after a dose and continuing lung assessment. Current cisplatin product information additionally advises weekly kidney/liver tests, blood counts and electrolytes through treatment, with repeat hearing testing for symptoms. Review nerves and bowel function before vincristine. Brain/spine imaging and any other response tests follow the individual specialist plan; no universal scan or tumour-marker timetable is set here.

STEP 7

Help with the daily plan

Agree help with prescriptions, travel, calls and symptom notes if fatigue, memory, sight or weakness makes these difficult. Your preferences guide who is involved. Keep the printed plan and real urgent contacts together.

Taking treatment and organising visits

Different medicines and support plans have different days.

Missed, vomited or held medicine

Medicine and supplement review

Safety tests, support and urgent symptoms

A treatment-free day is still part of the care plan.

Anti-sickness plan checked for interactions

Cisplatin has high emetic potential; lomustine adds oral moderate-to-high risk. Use the team's prevention and rescue plan, including delayed nausea after Day 1. Aprepitant and some antifungals interact with vincristine, so the oncology pharmacist must select and check the plan; do not add medicines yourself.

Hydration and electrolytes

Cisplatin needs fluids before and after treatment and an individual electrolyte plan. Tell the team about fluid limits, swelling, heart or kidney conditions, vomiting and reduced urine. Follow your own home-drinking advice rather than a universal volume.

Prevent constipation and monitor nerves

NCCP recommends preventive laxatives with vincristine. Report constipation, new tingling, weakness, balance or hand-function changes before each visit. Severe abdominal pain, vomiting or inability to pass stool or wind needs urgent assessment.

Medicines, infection prevention and recovery

Have all medicines checked, including seizure drugs, steroids, antibiotics, antifungals, non-prescription products and supplements. The team will confirm the local antifungal prevention plan and choose medicines that avoid vincristine interactions. Other infection prevention follows the individual specialist plan.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 42: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Lomustine · one oral dose	 **Day 1** Take once on Day 1 only; use the newly authorised prescription.
Cisplatin + prescribed hydration	 **Day 1** Day 1 with anti-sickness medicines, fluids and electrolyte plan.
Vincristine infusion	 **Day 1** Report bowel and nerve symptoms before this visit.
Vincristine infusion	 **Day 8** Report bowel and nerve symptoms before this visit.
Vincristine infusion	 **Day 15** Report bowel and nerve symptoms before this visit.
Continuing blood-count checks	 **Days 1–42** Lomustine SmPC advises weekly counts for at least six weeks; arrange all checks.
Symptoms and support between visits	 **Days 1–42** Keep the written emergency plan and do not wait for the next treatment to report changes.

A treatment record is not a prescription: The team may delay, alter or stop a component after reviewing symptoms, blood tests, organ function and previous toxicity. Do not restart a held medicine or add a replacement dose yourself.

Lomustine is one dose, not daily: Only the authorised Day 1 dose belongs to this cycle. Do not take a further course before six weeks and a new team decision. Taking repeated daily doses can cause serious harm. If a dose is missed or vomited, contact the team or pharmacist before taking any more. If an extra dose is taken, seek urgent clinical advice straight away even if you feel well; do not wait for symptoms.

Delayed marrow effects: Current lomustine product information advises blood counts weekly for at least six weeks after a dose. Low counts may be delayed and cumulative, so keep checks even when you feel well or the next treatment is postponed.

> **Why these bands stay broad:** Days without a scheduled dose still need symptom monitoring and planned tests. The calendar does not replace a treatment decision. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00806 v2 · current regimen ↗

> **There are no guaranteed "safe days".** Fever, shivering or suddenly feeling unwell needs urgent advice, including on treatment rest days. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. Follow the emergency instructions in any established seizure rescue plan.

Between-treatment notes

Day 1: Three medicines and hydration

Lomustine once by mouth, cisplatin and vincristine by infusion. Day 1 includes the individual fluid and anti-sickness plan. Confirm hearing, kidney, bowel and nerve symptoms before treatment.

Day 2–15: Vincristine visits on Days 8 and 15

There are no more scheduled lomustine or cisplatin doses in this cycle. Keep the bowel plan and report tingling or weakness. A held dose needs a new team decision; do not replace it yourself.

Day 16–42: Recovery and continuing tests

Counts can fall after infusion visits finish. Keep weekly lomustine blood-count checks and the team's other appointments. Call for fever, bleeding, new respiratory symptoms or difficulty drinking.

Support around the medicines

1 **Hydration and electrolytes**

2 **Medicines, infection prevention and recovery**

Understanding your particular brain tumour

CLV here is for selected adult high-risk medulloblastoma or another eligible embryonal tumour; it is not a glioma regimen.

The exact diagnosis matters

Medulloblastoma is an embryonal tumour that usually starts in the cerebellum. The NCCP regimen also names other primitive neuro-ectodermal tumours, including pinealoblastoma. Ask the specialist to record the current pathology name and explain any older wording. These diagnoses are not interchangeable with glioblastoma or other gliomas.

Risk group and spread

The team considers the exact tumour, what remains after surgery and whether it has spread within the brain, spine or cerebrospinal fluid. Ask what makes your tumour high risk and which results establish that. A general glioma grade explanation does not replace medulloblastoma risk assessment.

Molecular results

Medulloblastoma can be classified by molecular findings, including WNT, SHH and non-WNT/non-SHH groups. Ask which tests apply to your exact tumour and how they affect discussion. Molecular tests used for another tumour type are not automatically relevant to this diagnosis. If a result is pending or not applicable, record that clearly.

How the treatments fit together

Surgery obtains tissue and removes tumour where safely possible. Craniospinal radiotherapy treats planned areas of the brain and spine. This NCCP CLV schedule starts four to six weeks after craniospinal RT, after recovery and safety review. The team confirms the role of each part for your diagnosis.

A useful diagnosis record

Keep the pathology date and wording, risk group and its reasons, molecular results if relevant, surgery and RT dates, brain/spine scan findings and any cerebrospinal-fluid assessment. Ask who will explain pending results. Use this record to understand decisions, not to determine your own chemotherapy eligibility.

Tests, scans and recovery after treatment

Safety tests and tumour-response tests have different purposes.

Bloods, kidneys and hydration

Blood counts check marrow recovery; kidney and liver tests help assess whether treatment is safe. Cisplatin also needs an individual hydration and electrolyte plan. Tell staff about fluid restrictions, swelling, heart or kidney problems and difficulty drinking. Do not use a generic home fluid target in place of your plan. Current cisplatin product information advises weekly blood-count, kidney/liver and electrolyte checks through treatment.

Hearing, nerves and lung assessment

Cisplatin can affect hearing and nerves; vincristine can affect nerves and bowel function; lomustine can affect the lungs. Tell the team about changes before every visit. NCCP lists hearing and lung tests according to clinical need, while current lomustine product information advises baseline and continuing lung assessment. Ask which checks are booked for you. Current cisplatin product information advises a hearing test before treatment and repeat testing if hearing symptoms develop.

Brain and spine review

Medulloblastoma may involve the brain, spine or cerebrospinal fluid. Your specialist decides which imaging or other tests are needed at each stage. Keep the next scan date and result-review appointment in your passport. This guide does not set a universal scan, lumbar-puncture or blood tumour-marker timetable.

Delayed effects and rehabilitation

Symptoms may change after surgery, RT or chemotherapy. Hearing, balance, strength, memory, fatigue and daily function deserve follow-up as well as scans. Ask which longer-term checks and rehabilitation are relevant after your particular treatment. Report new changes promptly rather than waiting for a routine appointment.

Keep results with decisions

Record when a test is booked, completed and explained, and the next agreed step. A normal result from one test does not replace another medicine-safety check. The team authorises treatment after considering symptoms, current results and earlier side effects.

Brain symptoms, medicines and when to call

Symptoms may reflect the tumour, swelling, treatment, another medicine or a separate illness.

Know your usual symptoms

Before treatment, note your usual walking, strength, speech, sight, memory, sleep and seizure pattern. Record new changes and their timing. A trusted person can help if you want them involved.

Steroids need a separate written plan

Steroids may reduce swelling around a brain tumour. If needed, ask for a written dose, review date and reduction plan; do not stop suddenly or adjust the dose yourself. Report infection symptoms, marked thirst, weakness or troublesome sleep or mood changes.

Seizure medicines and rescue instructions

Take prescribed seizure medicine on its own schedule. Ask what to do after a missed or vomited dose and how to manage a breakthrough seizure. Do not switch or stop it yourself. Anyone giving a rescue medicine needs the personal instructions and training.

Safety at home and outside

Discuss driving, work, bathing, swimming, heights and machinery with your team, taking account of symptoms and seizures. Individual medical and licensing advice is needed. Occupational therapy can help make daily routines safer.

Your overall treatment plan and choices

Treatment should account for the tumour and the life you want to lead.

Understand the sequence

Place biopsy or surgery, radiotherapy, medicines and follow-up on one plan. Concurrent means treatments overlap; adjuvant means treatment follows the main treatment. A new phase can require different doses, tests and support medicines.

Agree what treatment is trying to achieve

Ask what treatment can reasonably achieve for your tumour and which effects matter most to you. Discuss benefits, burdens, alternatives and the reasons for a pause or stop.

Put evidence in context

A study result applies to a particular group, diagnosis and schedule. Older studies may use different tumour names. Ask which evidence fits your plan; an average cannot forecast your response or lifespan.

Discuss options available to you

Ask about suitable trials, another specialist opinion and other treatment or symptom-care options. Your multidisciplinary team explains what is appropriate and available for you.

Make room for your priorities

Bring up thinking, independence, family time, work and the side effects you most want to avoid. Ask for time, written notes or a supporter if that helps you weigh the choices.

Living well, rehabilitation and support

Practical and symptom support can begin alongside anticancer treatment.

Thinking, communication and daily tasks

Tell the team about changes in memory, language, attention, sight or planning. Rehabilitation can involve physiotherapy, occupational therapy, speech and language therapy or psychology. Ask for practical help with routines and medicines.

Food, nausea, bowels and energy

Use your prescribed nausea and bowel plan and report problems early. Ask for dietitian help if intake or weight falls. Adapt movement to your balance and strength, and ask the team to assess fatigue that disrupts daily life.

Emotional support and carers

Tell the team about changes in mood, behaviour or coping. Patients and carers may each need psychological or practical support. Agree who can receive information and ask for help before caring becomes overwhelming.

Work, transport and finances

Tell the nurse or social worker about travel, employment, finances and caring responsibilities. Arrange practical help with medicines or appointments if symptoms make those tasks difficult.

Supportive care and future wishes

Supportive and palliative care can help manage symptoms while treatment continues. When you are ready, discuss future care preferences and whom you want involved. Ask your team to help document these wishes.

Information that works on paper

Keep the latest paper schedule, medicine list, results plan and emergency contacts together. The treatment card helps others identify your regimen. It supports, but does not replace, the prescription and emergency instructions.

Questions worth asking

What is my exact diagnosis and how does this regimen fit?

NCCP 00806: selected adults with high-risk medulloblastoma or another eligible primitive neuro-ectodermal tumour, including pinealoblastoma. Ask for the pathology, relevant molecular findings and the treatment goal in plain language.

How do I handle a missed, vomited or held dose?

Contact the oncology team or pharmacist for medicine-specific instructions. Do not double a dose or replace treatment that was held for toxicity.

Who helps with steroids, seizures, hearing, memory or daily tasks?

Ask for written medicine plans and a named contact. Rehabilitation, practical support and a trusted helper may support the daily plan.

When do we review whether treatment is helping?

Agree the next clinical and imaging review, what would count as benefit, how side effects will be weighed and what would lead to continuing, changing or stopping.

> **Try explaining the plan back:** Can you show the day for each medicine, your next safety tests, the current support plan and whom to call for fever, new neurological symptoms or another urgent change?

CLV · 42 days NCCP 00806: selected adults with high-risk medulloblastoma or another eligible primitive neuro-ectodermal tumour, including pinealoblastoma. Use the exact prescription and schedule agreed with your specialist.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · CLV, hydration and antiemetic plan

Active medicines: Lomustine + Cisplatin + Vincristine

Emetogenic risk: high

Medicine	Dose / route	When
Individual high-risk antiemetic prevention and rescue	Agent / dose / duration to be completed by the prescriber and oncology pharmacist	Day 1 and delayed phase according to the agreed local regimen Incomplete review row; not a prescription. Pharmacy must reconcile aprepitant/ CYP3A4 and vincristine interactions; do not auto-populate a generic aprepitant bundle.
Cisplatin hydration and electrolyte replacement	Agent / dose / duration to be completed by the prescriber and oncology pharmacist	Before and after cisplatin; reassess fluid status and renal function Incomplete review row; not a prescription. Complete fluids, rates, magnesium/ potassium and monitoring for the actual product/local policy. No universal home fluid volume.
Constipation prophylaxis	Agent / dose / duration to be completed by the prescriber and oncology pharmacist	Start and continue on the individual vincristine bowel plan Incomplete review row; not a prescription. Select laxative and escalation instructions; review bowel symptoms before each vincristine dose.

Additional phase-specific support

- Cisplatin has high emetic potential; lomustine adds oral moderate-to-high risk. Use the team's prevention and rescue plan, including delayed nausea after Day 1. Aprepitant and some antifungals interact with vincristine, so the oncology pharmacist must select and check the plan; do not add medicines yourself.

- Cisplatin needs fluids before and after treatment and an individual electrolyte plan. Tell the team about fluid limits, swelling, heart or kidney conditions, vomiting and reduced urine. Follow your own home-drinking advice rather than a universal volume.
- NCCP recommends preventive laxatives with vincristine. Report constipation, new tingling, weakness, balance or hand-function changes before each visit. Severe abdominal pain, vomiting or inability to pass stool or wind needs urgent assessment.
- Have all medicines checked, including seizure drugs, steroids, antibiotics, antifungals, non-prescription products and supplements. The team will confirm the local antifungal prevention plan and choose medicines that avoid vincristine interactions. Other infection prevention follows the individual specialist plan.
- Draft for clinician/pharmacist completion, not an issued prescription. Confirm agents, doses, timing, duration, interactions and rescue instructions with the current written plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Days 8 and 15 · vincristine only

Active medicines: Vincristine

Emetogenic risk: Minimal

Medicine	Dose / route	When
Constipation prophylaxis	Agent / dose / duration to be completed by the prescriber and oncology pharmacist	Continue / adjust the written bowel plan during vincristine treatment Incomplete review row; not a prescription. Review constipation, ileus symptoms, peripheral/autonomic neuropathy and drug interactions before dosing.

Additional phase-specific support

- NCCP recommends preventive laxatives with vincristine. Report constipation, new tingling, weakness, balance or hand-function changes before each visit. Severe abdominal pain, vomiting or inability to pass stool or wind needs urgent assessment.
- Have all medicines checked, including seizure drugs, steroids, antibiotics, antifungals, non-prescription products and supplements. The team will confirm the local antifungal prevention plan and choose medicines that avoid vincristine interactions. Other infection prevention follows the individual specialist plan.
- Do not automatically repeat cisplatin hydration or its high-risk antiemetic course on vincristine-only days. Prior symptoms and the local individual plan can require additional support.
- Draft for clinician/pharmacist completion, not an issued prescription. Confirm agents, doses, timing, duration, interactions and rescue instructions with the current written plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Brain / hearing

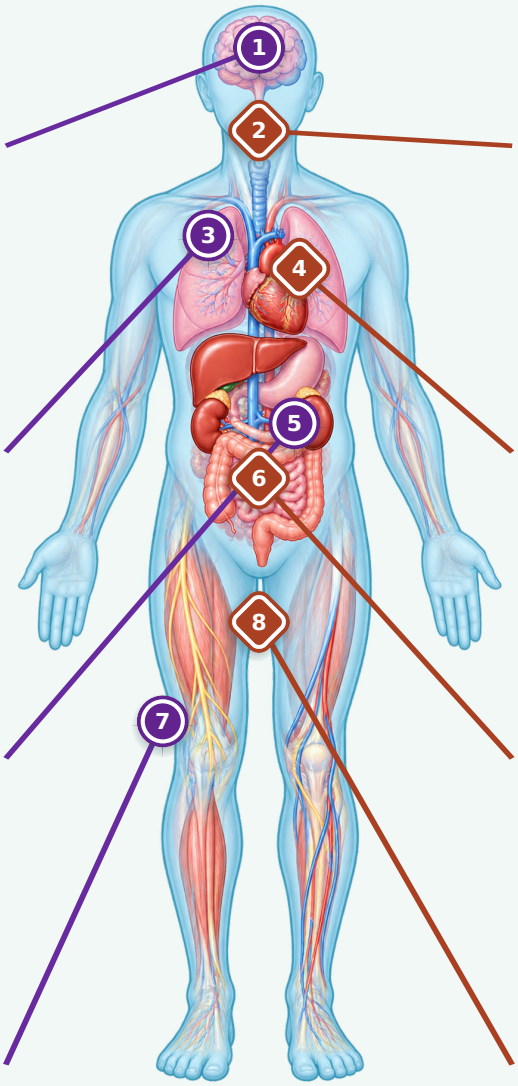
Cisplatin / tumour

Report new ringing or reduced hearing promptly. New seizure, sudden weakness or speech difficulty needs emergency help.

2 Mouth / eating

Chemotherapy

Soreness or ulcers can limit intake. Use gentle mouth care and ask for help before eating or drinking becomes difficult.



3 Blood / infection

Chemotherapy

Counts can fall weeks after lomustine. Fever, sudden illness, unusual bruising or bleeding needs urgent advice.

4 Lungs

Lomustine

Report new cough or breathlessness. Keep the lung checks arranged; severe breathing difficulty needs emergency help.

5 Kidneys / hydration

Cisplatin / lomustine

Attend kidney and electrolyte checks. Follow your individual fluid plan; report reduced urine, swelling or difficulty drinking.

6 Nausea / bowels

Cisplatin / vincristine

Use prescribed anti-sickness and bowel medicines. Severe pain, vomiting or no stool or wind needs urgent advice.

7 Hands / feet / nerves

Vincristine / cisplatin

Report tingling, numbness, weakness, balance or hand-function changes before the next dose; do not wait for them to worsen.

8 Fertility / pregnancy

Chemotherapy

Discuss preservation and contraception before starting. Do not breastfeed during treatment; ask for written precautions.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Use your oncology unit’s written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions. Call promptly for fever, bleeding, new cough or breathlessness, persistent vomiting or reduced urine. Report hearing, nerve or bowel changes before the next infusion.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital’s temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **Complete the individual local plan and interaction review before use; clinical/pharmacy approval pending**
   https://healthservice.hse.ie/documents/6870/806_V2_CLV_Therapy.pdf
2. **NCI: medulloblastoma diagnosis, risk groups and treatment · patient education**
   https://www.cancer.gov/rare-brain-spine-tumor/tumors/medulloblastoma
3. **HPRA: cisplatin Accord current product information · July 2026**
   https://assets.hpra.ie/products/Human/27566/Licence_PA2315-081-001_10072026160843.pdf
4. **Lomustine medac current product information · January 2026**
   https://www.medac.eu/fileadmin/user_upload/medac-eu/SPCs/common_SPCs/Lomustine_medac-spc-common.pdf
5. **NCI: adult central nervous system tumours · diagnosis and grade education**
   https://www.cancer.gov/types/brain/patient/adult-brain-treatment-pdq
6. **NHS: dexamethasone · use and stopping safely**
   https://www.nhs.uk/medicines/dexamethasone-tablets-and-liquid/how-and-when-to-take-dexamethasone-tablets-and-liquid/
7. **Packer 1994: childhood medulloblastoma study cited by NCCP · not an adult survival estimate**
   https://pubmed.ncbi.nlm.nih.gov/7931615/
8. **EANO guideline: diffuse gliomas of adulthood, 2021**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC7904519/
9. **NCI: fatigue during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/fatigue
10. **NCI: eating and nutrition during cancer treatment**
    https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
11. **NCI: palliative care during cancer treatment**
    https://www.cancer.gov/about-cancer/advanced-cancer/care-choices/palliative-care-fact-sheet